



February 23, 2026

Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology
Department of Health and Human Services
Mary E. Switzer Building
330 C Street S.W.
Washington, DC 20201

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care

Dear Secretary Kennedy,

Fresenius Medical Care (FME) appreciates the opportunity to comment on the Request for Information (RFI) from the Department of Health and Human Services (HHS) Office of the Deputy Secretary in collaboration with the Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology (ASTP/ONC) seeking feedback on policies to accelerate the adoption of AI in clinical care. FME is the largest integrated supplier in the country of services and products for individuals undergoing dialysis due to end-stage renal disease (ESRD). Operating over 2,600 outpatient renal dialysis facilities, FME provides dialysis services to more than 200,000 people with kidney failure in the United States each year. In addition, we are the leading dialysis medical supply and equipment manufacturer, and we develop and deploy technologies that leverage machine learning to enable real-time care optimizations that streamline care, assist clinicians, and ensure that life-sustaining dialysis treatments are tailored to each patient.

As a leader in dialysis-related AI technology, we view artificial intelligence as transformative in improving the quality, safety, and efficiency of care for our patients. For instance, we are actively exploring AI applications to streamline patient referrals and reduce documentation burden, predict clinical complications, optimize dialysis treatment outcomes through real-time monitoring, and accelerate medical research for dialysis care.¹ However, the successful integration of these advancements relies upon the safe deployment of AI tools built on reliable, high-quality data, supported by clear frameworks that outline the responsibilities among AI users including clinical care teams. Furthermore, a healthcare ecosystem that fosters the safe adoption of innovative AI requires a reimbursement infrastructure that appropriately compensates providers and incentivizes manufacturers to invest in these transformative technologies.

Executive Summary

We have structured our feedback around the three strategic pillars identified by HHS, with a specific focus on the unique needs of the ESRD population:

¹ Renal Research Institute. (2025). *Research Innovations*. <https://renalresearch.com/research/research-innovations/>

- **Regulation:** We recommend HHS update the ESRD Conditions for Coverage to account for the unique, nurse-led nature of dialysis care and clarify how AI-enabled recommendations may be incorporated into standing orders under appropriate medical direction. We recommend ASTP/ONC strengthen interoperability requirements by mandating FHIR-based APIs that support dialysis-specific data elements. Finally, we recommend HHS update HIPAA guidance to clarify that limited Health Information Exchange (HIE) data may be used for AI training and validation, with appropriate privacy safeguards in place.
- **Reimbursement:** We recommend that CMS align payment incentives by allowing dialysis providers to participate in "shared savings" generated by AI tools that, for example, reduce hospitalizations and emergency department visits. Additionally, CMS should establish reimbursement pathways for high-value Software-as-a-Service (SaaS) and AI-driven clinical decision support tools that fall outside the traditional dialysis bundle.
- **Research & Development:** HHS should invest in public-private partnerships that establish open, de-identified benchmarking datasets and fund multi-site validation studies to ensure AI tools are safe and equitable across diverse patient populations. Furthermore, HHS should promote using historical treatment data to rigorously test model performance and safety.

Our responses to HHS' detailed questions are below.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Data and Technical Barriers

Incomplete data and technical fragmentation can impede the development and clinical adoption of dialysis-specific AI technologies. High-quality, comprehensive training data is essential for developing AI tools that can reliably detect risk, personalize treatment, and support clinical decision-making. Because of the varied nature of dialysis care, longitudinal data for patients with ESRD is dispersed across nephrology practices, hospitals, skilled nursing facilities, transplant centers, and dialysis clinics, and no single entity maintains a comprehensive clinical profile. Consequently, AI models trained on isolated EHR data frequently overlook critical hospitalizations, medication changes, and acute events that fundamentally drive patient outcomes. This challenge is compounded by a lack of structured data within EHRs. Essential clinical indicators like patient symptoms, functional status, caregiver support, and dialysis or medication adherence challenges are often relegated to free-text fields or omitted from EHR data entirely. Furthermore, limited interoperability between dialysis centers and hospital systems results in inconsistent or delayed data exchange, frequently leaving providers without access to real-time information unless they are members of a specific Health Information Exchange (HIE).

Infrastructure gaps also persist within Encounter Notification Service (ENS) feeds, which vary significantly in quality and timeliness across jurisdictions. Although the frequent follow-up care required by the ESRD population presents a unique opportunity for coordination, inpatient facilities are not currently mandated to notify dialysis centers of admissions, which is a stark contrast to the notification



requirements already in place for primary care providers.² Finally, while Fast Healthcare Interoperability Resources (FHIR) application programming interfaces (APIs) and the United States Core Data for Interoperability (USCDI) standards are vital for interoperability, they are primarily designed for mainstream hospital settings and often do not include critical dialysis-specific data elements. Although dialysis providers share extensive treatment data through Continuity of Care Documents (CCDs), hospitals often do not reciprocate because this information falls outside current United States Core Data for Interoperability (USCDI) standards, undermining the continuity of care. Ensuring that AI is both safe and clinically valuable will require HHS to support the development and exchange of dialysis-relevant data elements that reflect the realities of patient care.

Regulatory and Reimbursement Barriers

Regulatory and reimbursement barriers also limit the development and use of AI in dialysis care. The ESRD Prospective Payment System (ESRD PPS) has not been modernized to create incentives to adopt high-value, safe AI that improves clinical outcomes. Dialysis organizations do not share any savings generated when AI reduces hospitalizations or improves care coordination. Further, the ESRD Quality Incentive Program (QIP) functions solely as a penalty program, offering no rewards for clinics that use AI to drive measurable improvements in patient care. Finally, the fully bundled structure of the ESRD PPS does not allow for differential reimbursement for AI-enabled technologies, limiting providers' ability to invest in tools that could enhance outcomes, and CMS has likewise not established a reimbursement framework for software-as-a-service tools within the ESRD payment system, even though similar mechanisms exist in other areas of Medicare payment policy.

Dialysis relies on standing orders for anemia, fluid management, and mineral bone disease. Introducing AI algorithms into these pathways raises questions about when a tool qualifies as a medical device and who is responsible when clinicians act on AI-supported recommendations. Furthermore, HIEs serve as a key information highway that dialysis providers rely on to receive timely hospitalization alerts, discharge summaries, laboratory results, and other clinical updates needed to manage fluid status, infections, and care transitions. These exchanges also function as valuable repositories of longitudinal clinical data that can serve as the basis for developing high-quality AI models tailored to the needs of ESRD patients. However, constraints on data use beyond treatment, payment, and operations restrict the ability to use HIE data for AI, research, or broader quality improvement activities. The high cost and uneven access to HIE and ENS further limit the availability of timely hospitalization data, particularly for smaller hospitals and dialysis facilities that cannot afford participation fees. These data limitations reduce the completeness and reliability of the underlying data, making AI tools harder to implement and less accurate for the ESRD population.

Operational and Workforce Barriers

Operational and workforce barriers also limit the feasibility of AI adoption in dialysis care. Nurse workload and persistent staffing shortages mean that any AI system adding tasks, alerts, or documentation requirements is unlikely to be workable in a setting where nurses simultaneously manage multiple patients throughout treatment. We caution tools that increase cognitive or administrative burden risk reduction, rather than enhancing, clinical efficiency. Furthermore, physician

² 42 CFR 482.24(d)(5)

presence in dialysis clinics is episodic, as outpatient nephrologists typically round intermittently rather than remaining on site. As a result, AI must be designed to support asynchronous, distributed decision-making rather than real-time physician oversight. Most regulatory frameworks, however, assume continuous physician availability, which does not reflect the current outpatient dialysis care model and may complicate the deployment of AI tools in this environment.

- 2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.**

Update ESRD Conditions for Coverage to Clarify Role of AI

HHS should update the ESRD Conditions for Coverage to clarify how AI-enabled tools can be appropriately integrated into dialysis standing orders and clinical workflows. As noted above, dialysis care relies heavily on standing orders that guide anemia management, fluid management, and mineral bone disease treatment, yet current regulations do not address how AI-generated recommendations fit within these established protocols. To support safe and effective adoption, HHS should clarify the circumstances under which AI-enabled tools may be incorporated into standing orders remain within the scope of required “medical direction,” what documentation and oversight obligations apply when nursing staff follow AI-assisted protocols, and how to distinguish between clinical decision support tools and regulated medical devices in dialysis settings. Clear regulatory guidance in these areas would reduce uncertainty for providers and manufacturers while ensuring that AI is deployed in a manner consistent with patient safety and existing requirements.

Modernize HIPAA for AI Training and Inference

HIPAA provisions addressing use of identifiable data were not designed to accommodate AI training or inference and were developed before current AI use cases emerged. The HIPAA Privacy Rule does not clearly address whether longitudinal data from HIEs or ENS feeds may be used to develop or refine AI models intended to improve care quality. HHS should update HIPAA guidance to clarify when and under what circumstance data obtained through HIEs and ENS feeds may be used for AI-enabled quality improvement activities, including the development and training of models that support care coordination, improve clinical outcomes and reduce healthcare costs. HHS should provide clear guardrails governing de-identification and use of limited data sets in AI training and inference contexts.

Align Payment Under the ESRD PPS

HHS should align payment incentives under the ESRD Prospective Payment System (PPS) to support the adoption of AI-enabled tools that improve clinical outcomes. This includes modifying the ESRD PPS to allow shared savings for reductions in hospitalizations, emergency department visits, or other potentially avoidable complications attributable to AI-enabled care coordination. HHS should also clarify payment policies for innovative software-as-a-service tools under the ESRD PPS, including those with embedded AI capabilities, consistent with the progress made in other Medicare payment systems.³

³ Medicare Payment Advisory Commission. 2024. "Paying for software technologies in Medicare." In *Report to the Congress: Medicare and the Health Care Delivery System*, 137-165. Washington, DC: MedPAC.

Considerations for ESRD CMMI Models

Future CMMI models for ESRD care should explicitly allow for AI-supported dialysis interventions that target key drivers of morbidity and cost. These models should incorporate, for example but not limited to, the use of AI for early detection of fluid overload, improved anemia management,⁴ vascular access creation and maintenance, reduction in intradialytic distress and optimization of hemodiafiltration dose, enabling more proactive and individualized care. Integrating these capabilities into model design would support innovation while advancing the goals of improved outcomes and reduced acute care utilization. CMMI has an opportunity to embed these capabilities into the next generation of ESRD models as the CKCC model sunsets in 2027.

Medicare Advantage

Recent congressional scrutiny^{5 6} has highlighted plan practices of using AI-driven prior authorization tools in ways that may delay or deny medically necessary care, including repeated or premature denials within the same episode of care, applying opaque internal criteria, and failing to account for changes in a patient's condition. We agree with other stakeholders who have called for stronger CMS oversight over MA plan use of AI to ensure transparency, accountability, and alignment with Medicare coverage standards so that AI is not used as a tool to further impede access to needed dialysis services.

ASTP/ONC Interoperability Frameworks

HHS should strengthen interoperability requirements to ensure dialysis organizations receive timely, actionable clinical information needed for safe and coordinated care. This should include requiring hospitals to provide real-time Admission, Discharge, and Transfer (ADT) notifications to dialysis organizations at no cost, establishing standardized ENS data formats and minimum content requirements, and mandating FHIR-based APIs that support dialysis-specific data elements such as treatment parameters, vascular access type, dry weight, and ultrafiltration rates.

- 3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?**

AI-enabled tools used in dialysis care raise several novel legal and operational issues. When AI systems inform clinical protocols carried out by nursing staff, there must be clear accountability frameworks to ensure that a qualified clinician reviews and validates all AI-generated recommendations. The dialysis

⁴ Garbelli M, Bellocchio F, Baro Salvador ME, Chermisi M, Rincon Bello A, Godoy IB, Perez SO, Shkolenko K, Perez AS, Toro DS, Apel C, Petrovic J, Stuard S, Barbieri C, Mari F, Neri L. The Use of Anemia Control Model Is Associated with Improved Hemoglobin Target Achievement, Lower Rates of Inappropriate Erythropoietin Stimulating Agents, and Severe Anemia among Dialysis Patients. *Blood Purif.* 2024;53(5):405-417. doi: 10.1159/000536181. Epub 2024 Feb 21. PMID: 38382484.

⁵ House Committee on Energy and Commerce. (2026, January 22). Health hearing: Lowering health care costs: An examination of health insurance affordability

⁶ House Committee on Ways and Means. (2026, January 23). Full Committee Hearing with Health Insurance CEOs.

population also presents heightened risks of model drift. Frequent hospitalizations, rapid changes in clinical status, and constant care transitions create a dynamic and evolving data environment where models built on historical patterns can degrade quickly without continuous monitoring and recalibration. In addition, because ESRD disproportionately affects Black, Hispanic, and low-income patients, safeguards are needed to ensure that AI tools do not amplify existing disparities in areas such as home dialysis access, transplant referral, or hospitalization risk prediction. HHS is well positioned to establish baseline expectations for clinical oversight, model monitoring, and bias mitigation to ensure that AI is deployed safely and equitably in high-risk care settings.

HHS can play an important role in clarifying the legal and operational expectations for AI use in clinical care. This includes issuing model governance guidance tailored to nurse-led, protocol-driven environments; developing standardized liability frameworks for AI-assisted standing orders; providing direction on how business associate agreements should address AI training, model updates, data provenance, and auditability; and encouraging HIEs to remove restrictive data-use limitations that impede safe and compliant model development and monitoring.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Promising pre-deployment evaluation methods for AI tools in dialysis care include multi-site retrospective validation across diverse patient populations to assess generalizability and safety; simulation using historical treatment data (such as ultrafiltration rates, intradialytic hypotension events,⁷ and hospitalization patterns) to test performance under realistic clinical scenarios; and structured human-factors testing with nurses and physicians to evaluate usability, workflow fit, and risks of misuse or over-reliance.

Post-deployment, AI tools should be supported by ongoing monitoring of key clinical and operational outcomes, including hospitalization rates, intradialytic hypotension, infection rates, and treatment adherence, along with drift-detection methods that flag performance changes associated with seasonal variation, staffing shifts, or changes in patient mix. Regular bias audits across race, ethnicity, disability, and geography, combined with human-centered and workflow evaluations that measure, for example, time saved, clicks reduced, handoffs improved, clinician burden reduction, and patient-facing outcomes such as comprehension of AI-informed decisions, perceived fairness, and trust, are essential.

HHS can accelerate the safe and effective use of AI in dialysis care by supporting rigorous evaluation across the full lifecycle of these tools. This includes funding targeted grants, cooperative agreements, contracts, and prize competitions that prioritize multi-site validation, human-centered design, and robust post-market surveillance in high-risk populations. HHS can further strengthen the evidence base

⁷ Hanjie Zhang, Lin-Chun Wang, Sheetal Chaudhuri, Aaron Pickering, Len Usvyat, John Larkin, Pete Waguespack, Zuwen Kuang, Jeroen P Kooman, Franklin W Maddux, Peter Kotanko, Real-time prediction of intradialytic hypotension using machine learning and cloud computing infrastructure, *Nephrology Dialysis Transplantation*, Volume 38, Issue 7, July 2023, Pages 1761–1769, <https://doi.org/10.1093/ndt/gfad070>

by supporting multi-center collaboratives that conduct dialysis-specific testing across diverse patient populations and care settings, as well as by enabling the development of open, de-identified benchmarking datasets for ESRD that incorporate relevant social risk variables.

A particularly important addition to the evaluation framework is the use of cluster randomized trials (CRTs) to assess clinical effectiveness and real-world impact. Because dialysis care is delivered in facility-based settings with shared workflows and team-based decision-making, randomizing at the level of the dialysis unit (rather than individual patients) is often the most practical and scientifically rigorous approach. Cluster randomized trials allow evaluation of patient-level outcomes (e.g., intradialytic hypotension rates, hospitalizations, infection rates, mortality, treatment adherence) while accounting for workflow integration, clinician behavior, and facility-level variation. They also reduce contamination bias that can occur when clinicians apply AI-informed practices to both intervention and control patients within the same unit. Embedding pragmatic CRTs into routine dialysis operations can generate high-quality causal evidence on safety, equity, effectiveness, and clinician burden in real-world settings, making them a critical component of deployment evaluation for AI tools in dialysis care.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Because the clinical use of AI in ESRD care is still rapidly evolving, HHS should be cautious about mandating specific certification programs before the underlying standards are mature and evidence-based. Premature accreditation and certification requirements could lock in frameworks that do not yet reflect real-world clinical workflows, equity considerations, or the unique needs of high-risk populations such as ESRD patients.

HHS should instead encourage the development of credible, third-party AI safety and effectiveness certification programs tailored to clinical use. These programs should focus on safety within clinician workflows, transparency of training data and model provenance, bias and equity testing, and reliable integration with EHR systems. To create meaningful incentives without imposing premature mandates, HHS can tie certification to preferential scoring in CMMI models and eligibility for shared-savings opportunities. Finally, federal guidance should support staged implementation to ensure safe and sustainable integration.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Where AI has Performed Well and Fallen Short

AI tools have performed well in domains where data are structured, abundant, and closely tied to pattern-recognition tasks. This includes imaging and signal interpretation, algorithmic dosing and other forms of optimized clinical decision support, ambient documentation tools that reduce clinician burden, and operational optimization such as staff or patient scheduling. These applications have generally met or exceeded expectations because they operate in well-defined workflows with clear outcome measures.

By contrast, AI has fallen short in areas that depend on complex clinical pathway integration. Even technically accurate models have had limited clinical impact (most notably for predictive risk scores) when deployed into fragmented data environments that lack complete, longitudinal, and interoperable data or workflows that cannot absorb additional alerts. Alert fatigue and workflow misalignment remain major barriers: tools that are accurate in isolation but poorly integrated into clinical routines are frequently overridden or ignored, limiting their effectiveness. These issues are exacerbated when the recommended clinical interventions are not well defined and clinical staff are not trained on how to interpret or act on the alerts. Moreover, clinicians should have a mechanism to provide feedback on alert usefulness or accuracy.

High-Potential Future Tools

Several categories of AI hold significant promise for improving outcomes and reducing costs in ESRD care and other high-risk populations. High-value opportunities include tools that integrate claims, EHR, social determinants, unstructured text, and patient-reported data to dynamically coordinate multi-condition care plans, including social services; AI for polypharmacy and deprescribing that identifies harmful interactions, unnecessary medications, and personalized deprescribing pathways; and tools that identify transplant opportunities and flag delays in referral. Additional high-impact opportunities include AI that predicts dialysis vascular access failure before thrombosis or infection occurs,⁸ and AI-augmented home and community-based care models that provide continuous monitoring and triage for frail, disabled, or high-risk patients.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Within healthcare organizations, the individuals with the greatest influence over AI adoption include the CEO, Chief Medical Officer, Chief Clinical Officer, Vice President of Nursing, and leaders in Legal, Compliance, Privacy, IT, and Cybersecurity.

Key administrative hurdles to AI adoption include lengthy procurement and contracting cycles that slow the evaluation and onboarding of new tools; limited clinic-level bandwidth and resources for workflow integration, training, and ongoing enhancements; and staffing shortages and competing regulatory burdens that reduce the capacity to adopt and sustain new technologies. Adoption is also constrained by the need for AI outputs that are straightforward, clinically appropriate, and usable within nurse-supported, protocol-driven care environments.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

⁸ Bellocchio, F., Titapiccolo, J., Saxena, S., Singh, T., Hippen, B., Maddux, F., Neri, L., & Nikam, M. (2025). WCN25-2743 The Predictive Performance and Usage of Artificial Intelligence (AI)-Based Fistula Failure Model in Singapore Hemodialysis (HD) Clinics. *Kidney International Reports*, 10(1), S67-S68. <https://doi.org/10.1016/j.ekir.2024.11.238>



Enhanced interoperability would significantly expand market opportunities and accelerate AI innovation by improving access to high-value data types that are currently fragmented or unavailable in standardized formats. Near real-time hospital admission, discharge and transfer feeds, home dialysis machine data, patient-reported symptoms and home weights, vascular access imaging and procedure data, social risk and caregiver support information, and unstructured clinical text are all critical inputs for building effective AI tools in ESRD care. Standardizing these data types and making them more readily exchangeable would enable more robust model development, validation, and deployment.

Interoperability gains would be further strengthened by advancing FHIR profiles for dialysis-specific data elements—including treatment parameters, access type, dry weight, and ultrafiltration rates—which remain inconsistently captured across systems. Standardized benchmarking tools built on these interoperable data sources would support transparent comparison of model performance, reduce barriers to entry for new developers, and fuel research that accelerates the development of clinically meaningful AI applications.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers consistently express interest in AI tools that help prevent avoidable hospitalizations, reduce treatment-related symptoms such as cramps, hypotension, and fatigue, and improve coordination between dialysis clinics, hospitals, and physician offices. They also want AI to support home dialysis and transplant by identifying upstream barriers earlier and providing clearer, actionable guidance. Further, patients value AI that helps deliver meaningful, personalized treatment plans that reflect their goals, preferences, and day-to-day realities.

Patients and caregivers also raise concerns about AI making decisions without their understanding or consent, and about the potential for bias to influence transplant eligibility or home dialysis recommendations. Data privacy is a major worry, particularly for vulnerable patients with limited digital literacy. Similarly, patients express fears that AI systems could misuse their data in ways that affect insurance coverage or employment. Many patients also express concern that AI could contribute to the dehumanization of care if not implemented thoughtfully.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

HHS should prioritize AI research that directly addresses the highest-impact clinical and operational challenges in ESRD care. Key opportunities include AI for fluid management and cardiovascular risk reduction, AI for infection prevention such as catheter- and access-related infections, and AI that strengthens home dialysis support and remote monitoring. Additional priorities include AI that improves transplant referral and evaluation pathways, reduces disparities in ESRD outcomes, and supports personalized, care for patients.

HHS should also invest in research that evaluates AI effectiveness and safety in real-world settings through pragmatic trials and quasi-experimental designs that measure actual outcome changes rather



than model accuracy alone.⁹ Finally, research on the economic and organizational impact of AI, including effects on workflows, staffing models, burnout, and cost structures, is essential to understanding the conditions under which AI can deliver its promised benefits.

a. Are there published findings about the impact of adopted AI tools and their use in clinical care?

Our teams have played a leading role in developing, evaluating, and implementing AI tools across multiple domains of kidney care, and we have included an appendix that highlights our published findings on the application of AI in ESRD care.

Conclusion

We appreciate the opportunity to respond to this RFI and look forward to collaborating with HHS to advance the safe implementation of clinical AI for dialysis patients.

Sincerely,

A handwritten signature in grey ink that reads 'Sarah Schmidt'.

Sarah Schmidt

SVP, Government Affairs

⁹ Kotanko P, Zhang H, Wang Y. Artificial Intelligence and Machine Learning in Dialysis: Ready for Prime Time? *Clin J Am Soc Nephrol*. 2023;18(6):803-805. doi:10.2215/CJN.0000000000000089

Appendix: Published Findings About the Impact of AI Tools on Dialysis Care

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